Session 4 Therapist Performance Review

Therapist: Jithendran Sellamuthu, C.MH., AHA #007913 | Client: TM-002 | Session 4: Nov 6, 2026 | Self-assessment + multi-perspective review | CONFIDENTIAL

OVERALL ASSESSMENT: B+ (Strong Session with Specific Growth Areas)

This was clinically the most significant session in the TM-002 case. The primary therapeutic objective (inner child reparenting targeting two core beliefs) was achieved. The client reported feeling "more complete," IMS confirmed subconscious agreement for protective part stand-down, and integration was confirmed via ideomotor nod. These are the outcomes that matter.

However, there are specific areas where the execution deviated from the prepared readout, and some clinical habits that -- if corrected now -- will compound into significantly better outcomes across your next 50+ clients.

WHAT YOU DID EXCEPTIONALLY WELL

1. Session Adaptation Without Losing Structure

For the fourth consecutive session, you adapted from the prepared script to follow the client's presenting material -- and for the fourth time, the adaptation was clinically correct. The uncle revelation (contradictory figure: supportive + frightening) was NEW data that emerged in-session. You recognized its significance immediately and integrated it into the therapeutic work without losing the session arc. This is the mark of a therapist who prepares thoroughly but holds the plan loosely. **This is your superpower. Protect it.**

2. Observer Canvas Execution

The screening room / externalization technique was delivered cleanly. Client moved from first-person emotional flooding to third-person observation. The color-draining / pixelation instruction created genuine psychological distance before the inner child contact. This is exactly what the Gemini 3-Phase model prescribed: desensitize BEFORE touching the wound. You followed the protocol.

3. Somatic Identification + Affect Bridge

You correctly identified that the belief is stored in the body ("psychosomatic physiology") and used the body sensation as a compass to trace back to the origin. This is textbook Nasir (neuroscience-informed) and it differentiates your approach from pure talk therapy. The client now has a framework for understanding WHY cognitive tools catch but cannot erase -- which reduces frustration with the process.

4. Secondary Gain Acknowledgment

Thanking the protective part before asking it to stand down was critical -- and you delivered it. Many therapists skip this because it feels counterintuitive ("why thank the thing causing the problem?"). But the subconscious built that firewall to survive. Asking it to dismantle without acknowledgment triggers resistance. Your delivery honored the system before updating it. The IMS finger lift confirmed the subconscious agreed.

5. "Don't Dilute the Feeling"

Instructing the client NOT to process verbally immediately after emergence -- to let the integration settle rather than intellectualize it -- is a subtle but expert move. Many therapists rush to debrief because THEY need to know what happened. You prioritized the client's subconscious processing over your own curiosity. **Keep doing this.**

6. Client Preference Respected

When the client said direct memory retrieval in past therapy worsened his anxiety for days, you immediately confirmed this matched your planned approach and deferred memory-retrieval work. You did NOT argue, educate, or subtly push. This builds trust faster than any technique. The client feels heard at the meta-level (how we work together), not just the content level (what we work on).

GROWTH AREAS (Honest Assessment)

1. SMART Baselines: 4th Consecutive Deferral

The transcript does not mention collecting SMART baselines (the six 1-10 scales). This is the **fourth session in a row** they have been deferred. The CCH log says "non-negotiable" for every session since S1. The CSP assessor flagged this in the clinical review.

Why this matters: Without pre-treatment baselines, you cannot objectively demonstrate post-treatment efficacy. When the CSP asks "did this client improve?" you need numbers, not narratives. "More complete" is meaningful clinically but it is not a measurable outcome.

Fix: Collect them in the FIRST 3 MINUTES of Session 5. Before the check-in. Before anything else. Script the exact words: "Quick baselines. Six areas. Just numbers. No right or wrong." This takes 90 seconds. There is no valid reason to defer again.

2. Readout Phases Skipped Without Documentation

The 22-phase enriched readout included several phases that do not appear in the transcript:

PHASE	WHAT WAS PLANNED	WHAT HAPPENED
13	Critical Voice Identification (whose voice says "not good enough"? Screen projection, phase out uncle's voice, replace with adult)	Not mentioned in transcript
14	Life Path Walk (hand-in-hand through abandonment experiences, smile and wave, forward to Tuesday group with thinner wall)	Not mentioned in transcript
16B	Mesh Filter PHA (LEFT hand for Tuesday group)	Not delivered (confirmed deferred to S5)
12	Bubble Pop metaphor + Beach Ball callback seal	Not mentioned in transcript

Why this matters: Some of these were gated (Phases 13, 14 -- only if time/capacity). Skipping gated phases is correct clinical judgment if the client was at capacity after the reparenting. But Mesh Filter PHA was a standalone install that could have been done in 2 minutes. And the beach ball callback would have bookended the session.

Fix: After each session, do a quick readout reconciliation: which phases were delivered, which were skipped, and why. Add this to the post-session checklist. The "why" is the clinical reasoning that justifies the adaptation. Without it, it looks like you forgot rather than chose.

3. Previewing Future Modalities During Deep Work

You explained cord cutting and cord realignment during this session. While client education is valuable, there is a clinical argument against previewing future techniques during or immediately after deep inner child work:

- It can create **anticipatory anxiety** ("cord cutting sounds intense -- am I ready for that?")
- It **dilutes the present session's impact** by shifting attention to what's next rather than what just happened
- It gives the conscious mind something to intellectualize, which can interfere with subconscious processing

Fix: Save technique previews for the CHECK-IN portion of the NEXT session, not the debrief of the current one. After inner child work, the debrief should be: "How do you feel?" + homework + safety. Period. Let the work settle. You said "don't dilute the feeling" -- then don't dilute it with a mini-lecture on cord cutting.

4. Reframing Fatigue -- Did You Explore It Deeply Enough?

The client reported that the protector energy reframe lasted ~1.5 weeks then felt "forced and numbing." You correctly identified this as evidence that behavioral tools cannot reach somatic beliefs. But did you explore the specific moment it shifted? What was happening in his life when the reframe stopped working? Was there a specific trigger that broke through?

Why this matters: The 1.5-week fade rate is a data point. If you track this after S4's inner child work, you can measure whether the deeper intervention extends the durability. If the integration holds for 3+ weeks, that is quantitative evidence of subconscious-level change. If it fades at 1.5 weeks again, the somatic reprogramming did not hold and you need a different approach.

Fix: In the post-session email or at S5 check-in, explicitly ask: "How long did the 'more complete' feeling last? Is it still there? If it faded, when exactly did you notice the shift?" Track the fade rate across sessions as a clinical metric.

5. Trance Depth Verification

The transcript does not mention any formal trance depth verification (e.g., finger-spread PHS verify, arm rigidity challenge, or depth self-report). You used IMS for the protective part stand-down, which is good. But you did not establish a depth baseline before entering the inner child work.

Why this matters: If the client was in light trance (alpha) rather than deep trance (theta), the inner child work may have been more imaginal/conscious than truly subconscious. The IMS finger lift could occur at light trance. Without depth verification, you cannot calibrate the suggestions appropriately.

Fix: After the staircase deepener and before the Observer Canvas, add a quick depth check: "On a scale of 0 to 10, where 10 is the deepest relaxation you have ever felt, where are you right now?" Or use arm catalepsy. This takes 15 seconds and gives you critical calibration data.

TECHNIQUE EXECUTION SCORECARD

TECHNIQUE	GRADE	NOTES
Emotional-lane Induction (affirmations + visual anchor)	A	Consistent with S2. Repetition = association. "Happy face" adaptation is strong.
Deepening (5-0 + staircase + progressive)	A	Full sequence delivered. Crown-to-toe with burden release framing.
Observer Canvas Desensitization	A	Screening room, color draining, pixelation. Clean execution.
Somatic Identification + Affect Bridge	A	Body location → compass → origin. Textbook.
Mist Metaphor + Child Meeting	A	Adult through mist, subconscious chose age. Agency language.
Inner Child Reparenting (2 beliefs)	A	Both beliefs addressed. Client discovered words. Not therapist-scripted.
Secondary Gain Acknowledgment	A	Firewall/watchtower honored before stand-down request.
IMS (Ideomotor Signaling)	A	Involuntary finger lift confirmed. Strong trance indicator.
Safe Room Construction	B+	Client-designed. Could use more sensory detail prompting (smell, temperature).
Integration	A	Embrace + nod confirmed. Deep breath to carry into cells. Clean.
Count Out	A	Staircase ascent, subconscious door closed. Gentle grounding.
SMART Baselines	D	4th deferral. Unacceptable. Must collect S5.
Critical Voice Replacement	C	Not delivered. Was gated -- but no documentation of why skipped.
Mesh Filter PHA	C	Not delivered. Deferred to S5. Reasonable but means Tuesday group has no new anchor.
Post-Session Debrief	B	"Don't dilute" was right. But cord cutting preview was premature.
Homework Assignment	A	Pre-sleep reparenting, recording, consistency. Clear and specific.

WHAT AN HMI INSTRUCTOR WOULD SAY

Lisa Machenberg

"The wound identification and reparenting delivery were strong. You correctly used the client's own words and let him discover the reparenting language rather than scripting it. The somatic identification piece is something most students skip -- you didn't. **But where are your baselines?** Four sessions without numbers. I need numbers. The client says 'more complete' -- what does that mean on a scale? How do I know the inner child work moved the needle if you never measured where the needle was? Collect them next session. First thing. Non-negotiable."

John Melton

"Good room technique execution. New room, client-designed, agency language, no compulsory merging. The child's age was subconsciously selected -- correct. You didn't lead the witness. **Watch the post-trance education.** You gave him a mini-lecture on cord cutting while he was still integrating. That's the conscious mind jumping back in too early. After emergence, it's: 'How do you feel?' Pause. 'Don't analyze it.' Homework. Safety. Done. Everything else goes in the email or next session's check-in. Let the subconscious have the night."

George Kappas

"You addressed the programming, not the story. Good. You moved from check-in to trance without dwelling in narrative. The uncle revelation could have derailed you into 20 minutes of biographical exploration -- you didn't let it. You took the data and went to work. **But you're still overbuilding your readouts.**

22 phases is a strategy document, not a session plan. A 90-minute session has room for 12-15 phases maximum. Build the full readout for preparation, then mark 5-6 phases as 'core' and the rest as 'if time/capacity.' Know what you will cut BEFORE you start."

Saman Nasir

"The affect bridge from somatic location to childhood origin is neuroscience-sound. You correctly explained psychosomatic physiology to the client -- that the belief is stored in tissue, not thought. This gives the client a framework for patience: 'I understand why thinking about it differently doesn't fix it.' **Add a depth verification** after deepening and before the therapeutic work. If you're doing memory reconsolidation, you need theta-level access. An arm catalepsy test or a self-reported depth score takes 15 seconds and tells you whether you're working at the right level."

TOP 5 ACTIONS TO IMPROVE

1. **Collect SMART baselines in S5 opening (90 seconds, zero excuses)** -- this is your #1 gap across the entire case. Everything else is strong. This is a documentation discipline issue, not a clinical skills issue.
2. **Add trance depth verification after deepening** -- 15-second self-report or arm catalepsy. Calibrates everything that follows. Becomes a data point you can track across sessions (is he going deeper each time?).
3. **Post-session discipline: debrief → homework → safety → stop** -- no technique previews, no education, no "next time we might do X." Let the work settle. Put the education in the email.
4. **Post-session readout reconciliation** -- after every session, spend 2 minutes noting which phases were delivered, which were skipped, and why. Add to the checklist. This is your clinical reasoning audit trail.
5. **Track the fade rate** -- explicitly ask at S5: "How long did the 'more complete' feeling last? When did you first notice a shift?" Compare to the 1.5-week fade rate from the protector energy reframe. This is your efficacy metric.

THE BIGGER PICTURE

You are 4 sessions into your second CSP client. You have delivered:

- EFT introduction (S1)
- Full emotional-lane induction with hypnodrama (S2)
- Psychoeducation with clinical history gathering (S3)
- Inner child reparenting with Observer Canvas, affect bridge, somatic identification, secondary gain acknowledgment, IMS, and integration (S4)

That is a sophisticated 4-session arc for a complex developmental trauma case. The client went from "6 vague presenting themes" to "more complete" with confirmed IMS and integration nod. The protector energy transferred to real life (work incident). The inner child work landed experientially, not just cognitively.

Your clinical instincts are strong. Your adaptation ability is rare for a student. Your preparation depth (22-phase readouts, multi-source technique integration, AI-assisted clinical analysis) is far beyond HMI standard.

The growth areas are all **discipline issues, not skill issues**: baselines, documentation, post-session boundaries, depth verification. These are the unsexy fundamentals that separate good therapists from great ones. The skills are there. The discipline catches up with practice.

Overall: You are ahead of where most HMI students are at this point. The clinical work is landing. Tighten the fundamentals and you will be formidable.